

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.046**

SUBJECT: Patient Falls Program

POINT PERSON: Chief Nursing Officer

APPROVED: Sara Walker, MD

DATE: FEBRUARY 18, 2025

Interim Superintendent

SELECT ONE: ☐ New policy

☐ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. This Policy and Procedures establishes expected response to patient falls and subsequent care and preventative measures to reduce risk of falls at Oregon State Hospital (OSH).
- B. This policy applies to all staff excluding students, volunteers or vendors unless properly supervised or trained.

II. POLICY

- A. Every patient shall be assessed for risk of falling via a Fall Risk Assessment per Nursing protocol 2.200 "Fall Risk Assessment" as described in Procedures A.
- B. A patient who falls must be appropriately addressed, evaluated, and monitored. For response to a fall and fall risk reassessment, see Procedures A.
- C. If any member of the IDT or direct care staff observes a potential change in patient status that may impact their fall risk, they should communicate this to the RN. The RN must determine if a reassessment of the fall risk is needed, justification for reassessment or not conducting a reassessment must be documented in avatar.
- D. Staff must monitor the patient care environment and will reasonably address potential fall hazards.
- E. The Falls Committee is an interdisciplinary group of fall prevention subject-matter experts whose primary goals are to promote patient safety, understand and

mitigate risks, reduce pre-/post-fall risks, improve patient outcomes, and provide training and awareness.

- F. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- G. Failure to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. “Fall” means a sudden, independent movement to the ground or lower surface that is unintentional, uncontrolled, or involuntary. It may be witnessed or not witnessed, with or without injury. A fall resulting from an aggressive interaction with another individual is not included in this definition. (NOTE: although fall monitoring processes would apply, such falls are reported separately.)
- B. “Staff” includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

Procedures A Fall Risk Assessment, Response to a Fall and Reassessment

V. HOSPITAL RESOURCES

Patient Fall Prevention and Interventions OWL page

VI. RELATED OSH POLICIES AND PROTOCOLS

Nursing Protocol 2.200 Fall Risk Assessment

Nursing Protocol 2.205 Post Fall assessment and Monitoring

VII. REFERENCES

42 CFR § 482.13(e).

42 CFR § 482.41(a).

42 CFR § 482.62(d).

Joint Commission Resources, Inc. (2025). The joint commission comprehensive accreditation manual for hospitals. HR.01.05.03. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2025). The joint commission comprehensive accreditation manual for hospitals. PC.01.01.01. – PC.01.02.08. Oakbrook Terrace, IL: Author.

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Joint Commission Resources, Inc. (2025). The joint commission comprehensive accreditation manual for hospitals. Sentinel Event Policy. Oakbrook Terrace, IL: Author.